7.5 Acute Kidney Injury*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I am passing very little urine." **Objective** • Urine 15 mL/hour for 6 hours • Creatinine rising • Crackles and edema	**ACTUAL** Excessive fluid volume related to reduced renal excretion as evidenced by oliguria, edema and crackles.	• Fluid balance achieved and weight steady.	• Measure urine hourly and keep a strict intake and output record. • Weigh daily. • Restrict fluids as prescribed, often the previous day's output plus insensible loss. • Go through nephrotoxic drugs with the physician. • Prepare for dialysis if ordered.	• Outcome reached when fluid balance is achieved.
Subjective • "I feel weak and my heart is jumping." **Objective** • Serum K^+ 6.4 mmol/L • Peaked T waves on ECG	**ACTUAL** Decreased activity tolerance related to muscle weakness and cardiac conduction changes associated with hyperkalemia as evidenced by reported weakness, palpitations and ECG changes.	• Serum K^+ below 5.5 mmol/L with a normal ECG.	• Keep him on continuous ECG monitoring. • Give calcium gluconate, insulin–glucose and other treatment as prescribed. • Restrict dietary potassium. • Repeat potassium as ordered.	• Considered met once K^+ is normal.
Subjective *No specific complaint* **Objective** • Uremia • Central line and urinary catheter	**RISK** Risk for infection as evidenced by risk factors: uremia and invasive devices.	• No infection.	• Use aseptic technique for line and catheter care. • Follow temperature and WBC. • Remove devices once they are no longer needed.	• Goal reached if no infection develops.

*References for this care plan: [35], [42], [85], [128]. Full entries are listed in the References section of the CPMN book.